The following is an example of a real-world case involving a patient with Dengue disease and having a drug induced adverse drug reaction (ADR), written for Task 1: ADR Identification Basics.

Patient and Reaction Description

- Patient information: 34-year-old female patient presenting with high-grade viral fever, joint pains, and diagnosed with Dengue infection (NS1 positive).
- Reaction description: On day 4 of hospitalization, the patient presented with acute onset of widespread erythematous rashes with intense pruritis and elevated liver enzymes (AST/ALT).
- Timeframe of development of adverse effects: Symptoms occurred approximately 72 hours post initiation of oral Cefixime and high dose paracetamol treatment.

Medication History

Here is the table, ready to copy. It pastes as a proper table into Word, Excel, or Google Docs.

Drug name	Dosage and route	Frequency	Indication	Start date	End date	Status
Paracetamol	1000 mg (Oral)	Every 6 hours	Fever and body aches	Day 1	Day 4	Dechallenged (Stopped)
Cefixime	200 mg (Oral)	Twice per day	Empirical coverage	Day 1	Day 4	Dechallenged (Stopped)
IV NS	100 mL/hr (Intravenous)	Continuous	Hydration	Day 1	Ongoing	Continued

Suspected Drugs, Temporal Relationship, and Dechallenge Results

1) Suspected drugs: Cefixime 200 mg (Primary suspected cause of the acute drug induced rashes) and Paracetamol 1000 mg (Secondary suspected cause of mild hepatotoxicity/elevated liver enzymes)
2) Temporal relationship: Rash and elevated liver enzymes occurred 3 days post initiation of Cefixime and high dose Paracetamol.
3) Dechallenge results: Discontinued on day 4 on observation of symptoms of drug induced rash and elevated liver enzymes.
Hydration with IV normal saline continues, rash subsides in next 48 hours.

Evaluation of the causality of drug induced adverse effect

- Classification: Probable
- Naranjo algorithm scoring:
  1. Temporal relationship between adverse drug reaction and drug administration – Positive

2. Symptoms improvement following drug discontinuation - Positive
3. Reactions cannot be attributable solely to the underlying disease process – Dengue infection causes petechial rash not erythematous pruritic rashes